

Policy on Management and Control of Data and Information

«Hospital Name»

Document control

Document No./Version	_____ / 1.0	Issue No./Review due	01 / One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	IT/MIS In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

Statement of intent

«Hospital Name» has processes in place for management and control of data and information, so collected data is standardised, analysed and reaches the right people accurately and on time.

1. Purpose

This policy sets out how «Hospital Name» standardises data collection, analyses data to meet information needs, disseminates information in a timely and accurate manner, stores and retrieves data, and involves clinical and managerial staff in using data.

This policy does not cover identifying information needs, medical record content, or confidentiality and security of records — those are covered in other hospital policies.

2. Scope

This policy applies to the collection, analysis, dissemination, storage and retrieval of all data and information at «Hospital Name».

3. Policy standards

«Hospital Name» standardises the process for data collection, including format and frequency.

«Hospital Name» analyses data to meet the information needs identified under Section 5.1 of the Policy on Meeting Information Needs.

«Hospital Name» disseminates information in a timely and accurate manner.

«Hospital Name» stores and retrieves data according to its information needs.

«Hospital Name» involves clinical and managerial staff in selecting, integrating and using data for meeting the information needs.

4. Non-negotiable rules

1. Do not collect data through unstandardised formats or frequency — process, format and frequency (event-based or defined periodicity) must be defined in advance.
2. Do not leave collected data unanalysed, or deny it the tools, techniques, and men/material/space/budget resources needed to meet the information needs it serves.
3. Do not disseminate information late or inaccurately to the stakeholders it is meant for.

4. Do not store data without adequate safeguards for its protection, or in a way that blocks timely access for patient care, education, research or service management.
5. Do not select, integrate or use data for meeting information needs without appropriate clinical and managerial staff participation.
6. Staff who see a rule under this policy broken report it the same shift to the IT/MIS In-Charge or the Medical Superintendent.

5. What we do

5.1 Standardised data collection

Processes for data collection at «Hospital Name» are standardised, covering both the format used — physical and/or electronic forms — and the frequency of capture, which may be event-based or follow a defined periodicity such as daily, weekly, monthly, quarterly or yearly, as appropriate to each data type.

5.2 Data analysis

Collected data at «Hospital Name» is analysed using appropriate tools and techniques to ensure the information needs identified under the Policy on Meeting Information Needs are met. Necessary resources — personnel, material, space and budget — are made available for data analysis.

5.3 Timely and accurate dissemination

«Hospital Name» gives timely and accurate information to relevant stakeholders after data analysis. The organisation decides which information needs to be shared with whom, and the modality used — for example memos, circulars or a webpage.

5.4 Storage and retrieval

«Hospital Name» stores data physically or electronically according to its information needs. Where electronic storage is used, adequate safeguards protect the data. Storage and retrieval systems at «Hospital Name» facilitate timely access to information for patient care, education, research and management of services.

5.5 Staff participation

Appropriate clinical and managerial staff at «Hospital Name» are responsible for selecting relevant indicators, measuring trends, and initiating action where required. A multidisciplinary committee may carry out this function.

6. Governance and responsibility

Role	Responsibility
IT/MIS In-Charge	Owns this policy. Maintains data-collection standards and storage/retrieval systems.
Multidisciplinary Data Committee	Selects indicators, analyses data trends, and initiates action as needed.
Medical Superintendent	Oversees implementation of this policy. Receives escalations under Section 4.
All Staff	Follow standardised data-collection formats and frequency. Report identified risks and rule breaches.

7. Quality monitoring

The IT/MIS In-Charge reviews data-collection standardisation, analysis resourcing, and dissemination timeliness on an ongoing basis. This policy is reviewed annually, or sooner if NABH standards change.

8. Training and staff acknowledgement

All staff covered by this policy are trained on the standardised data-collection formats and frequency relevant to their role, and sign the acknowledgement below.

Acknowledgement statement:

"I have read the Policy on Management and Control of Data and Information of «Hospital Name». I will follow the processes described."

Signature table (one row per staff member):

Staff Name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy is distributed to the IT/MIS department, all department heads/unit in-charges, and is made available to all staff through the hospital intranet and department policy folders.

10. Abbreviations

Abbreviation	Meaning
HCO	Hospital (Full Accreditation programme under NABH Hospitals 6th Edition)
NABH	National Accreditation Board for Hospitals and Healthcare Providers
IMS	Information Management System (NABH Hospitals 6th Edition chapter)
OE	Objective Element

11. Traceability to NABH HCO Full Accreditation 6th Edition — IMS.2

This table is an index. It is not how the process is organised. An asterisk in the Level column means documentation of the process is required.

OE	Level	Requirement	Where addressed	Responsible
IMS.2.a	Commitment	Data collection processes standardised	Section 5.1	IT/MIS In-Charge
IMS.2.b	Commitment	Data analysed to meet information needs	Section 5.2	IT/MIS In-Charge
IMS.2.c	Commitment	Information disseminated timely and accurately	Section 5.3	IT/MIS In-Charge
IMS.2.d	Commitment *	Data stored/retrieved per information needs	Section 5.4	IT/MIS In-Charge
IMS.2.e	Commitment	Clinical/managerial staff participate in data use	Section 5.5	Multidisciplinary Data Committee

12. Required Records / Evidence Checklist

IMS.2.a — Standardised data collection.

- Defined data-collection format and frequency record for each data type (event-based or periodic).
- Sample completed collection forms — physical and/or electronic.

IMS.2.b — Data analysis.

- Analysis-tool/technique record used per data type.

- Resource-allocation record — personnel, material, space, budget — for data analysis.

IMS.2.c — Timely and accurate dissemination.

- Dissemination record naming the stakeholder, information shared, and modality used (memo, circular, webpage).
- Timeliness-tracking record for dissemination against analysis completion.

IMS.2.d — Storage and retrieval.

- Storage-safeguard record for electronic data.
- Retrieval-access record demonstrating timely access for patient care, education, research and management.

IMS.2.e — Staff participation.

- Multidisciplinary committee constitution record, or named responsible clinical/managerial staff record.
- Indicator-selection and trend-measurement record, with action-initiated record where required.

13. References

- NABH Guidebook for Accreditation of Hospitals, 6th Edition — Standard IMS.2, "The organisation has processes in place for management and control of data and information."

Disclaimer

This document is a template prepared for the guidance of «Hospital Name» and must be reviewed, adapted and formally approved by «Hospital Name» before use. Every entry marked _____ must be completed before the document is issued.

The requirements in this document are accreditation requirements of the NABH Full Accreditation Standards for Hospitals, 6th Edition, not duties under a named Act of Parliament. Statutory duties that arise under other documents of «Hospital Name» remain those documents. «Hospital Name» is responsible for verifying any statutory duty that applies to it; this document does not constitute legal advice.

The clinical and technical content reflects recognised national and international guidance current at the date of preparation. «Hospital Name» remains responsible for verifying that it is current and consistent with the edition of the accreditation standard against which it is being assessed.

This document is not issued by, endorsed by, or affiliated with NABH or any other body named in it. Wording is original; no text has been reproduced from the standards, rules or guidelines referenced.